



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : امال فكرى محمود موسى
المحافظة : الجيزة
الحى / المدينة : العمرانية
العنوان : ==
رقم البطاقة : 25902060100347 رقم التليفون : 01068303550
العمر (تاريخ الميلاد) : 06-02-1959 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 23-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....)
المقرر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج هرموني خط ثاني مثبتات
الاروماتيز [14161]
قسم العلاج :
الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / / د. / / د. / / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: **امال فكري محمود موسى**
National ID: **25902060100347**



تقرير طبي

المسيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الطول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Second Line Hormonal Treatment Aromatase Inhibitor

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

09/16/2026

MDT FORM

Name: أمل فكري محمود موسى	MRN: 10885
Age: 68 Y	ID: 25902060100347
Menopausal Sta: post menopausal	Phone: * 01126291984
NRS SCORE:	Marital Status:
BMI:	Family History:
Gender:	Reason Of MDT:
CO Morbidity:	new case ,, on adj ET
Surgical History: * Bil MRM 11/2024	ECOG Performance Status
	☐ 0 ☒ 1 ☐ 2 ☐ 3 ☐ 4

Clinical Data:

BX: Mixed IDC, ILC G2, Same behaviour bilateral

ER: +ve 100% **PR:** 60% **HER2:** 0 **KI67:** 30%

History:
Bilateral Breast cancer (Synchronous) Rt cT1N1/ Lt cT3N3/Luminal B/
Received NACT ACx4 , TC x4 then Underwent Bil MRM--> PORT bil ,,
Now on adjuvant hormonal ttt (completed 2y of Femara)
Wishes to continue at our institute

Sonamammography:

Biomarker/Molecular

Mets Workup:

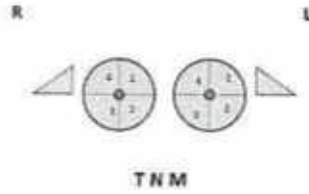
CT CAP in 6/2026 : free

Recommendations:

To continue adj Femara

MDT Outcome & Plan

ADJ FEMARA



Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. ندى حسن	د. محمد شوقي		



بدون عمل



النسبة

مسجلة

أنشئ

البطاقة سارية حتى ٢٠٢٨/٠٣/٢٢



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جمهورية مصر العربية
بطاقة تحقيق الشخصية

أمال

فكري محمود موسى

٣٦ شهر عبود الزمر

الجيزة - الجيزة

١٩٥٩/٠١

٢٥٩٠٢٠٦٠١٠٠٣٤٧

IU8914088

أ.د. هادي اسماعيل - أ.د. نيفين تاحون
د. حبيبة الشندي - د. م. جاد - د. ناسي حسن
د. مصطفى أحمد - د. محمد يسري - د. علي
د. محمد الشناوي - د. أحمد علي شحاتة

رئيس الوحدة
أ.د. محمد أكرم نوح

Pathology Unit

5287 السن : 65

تاريخ الحضور : 10-02-2024

تاريخ الاستلام : 12-02-2024

فرع : B-Mourad

إسم المريض : أمال فكرى محمود RT.B

محول من أ.د. محمد لبيب

رقم الزيارة : 53624500510

NATURE OF SPECIMEN:

Bilateral breast U/S guided core biopsies, for highly suspicious breast solid lesions, BIRDS 5.

GROSS:

Two biopsies were received:

- (1) Rt. breast: Five core range in length from 0.8-1.5 cm.
- (2) Lt. breast: Five core range in length from 0.9-1.6 cm.

MICROSCOPY:

(1) and (2) Both breast core biopsies show an invasive mixed breast carcinoma with areas of ductal and lobular differentiation. In areas tumor is formed of irregular groups of large malignant cells exhibiting focal ductal differentiation. In other areas tumor is formed of linear groups of small and intermediate sized cells with scanty cytoplasm. Stroma is moderately fibrotic with focal signet ring differentiation. Anaplasia and mitosis are moderate. Focal lymphovascular emboli are seen. Focal perineural is seen in (Biopsy 2).

DIAGNOSIS:

(1) and (2) Bilateral breast masses, US-guided core biopsies: Mixed invasive carcinoma of no special type, NST (= Invasive duct carcinoma) and lobular carcinoma, grade 2.

Neveen Tahoun

أحمد علي

Prof. Dr. Neveen S. Tahoun

Dr. Ahmed Ali Shehata

Authentication Date: 12-02-2024

أخصائى العينة بمعرفة الراسل وأخت مسؤلة العينة

Rt

بالتأكيد

ملح

أ.د. ندى اسماعيل - أ.د. ندى طه
 د. منية الفدي - د. علي خاد - د. ناسر حسن
 د. محمد علي أحمد - د. محمود بسري - د. علي الأيوبي
 د. محمد الشاوي - د. أحمد علي شحاتة

رئيس الوحدة
 أ.د. محمد أكرم نوح

Pathology Unit

B-Mourad فرع

6656 السن : 65

اسم المريض امال فكري محمود 00

تاريخ الحضور : 21-02-2024

محول من أ.د. محمد نجيب - شامير محي - شمال الصعيد

تاريخ الاستلام : 24-02-2024

رقم الزيارة 53624500677

NATURE OF SPECIMEN:

Paraffin block for immunostaining.

GROSS:

Sections immunostained for ER, PR, Ki67 and HER2 using Ventana BenchMark (Ultra) autostainer and Ventana Ultra View DAB detection system.

DIAGNOSIS:

ER Status : Rt. breast: Positive (95% of cells, intensity ++++)

PR Status : Rt. breast: Positive (70% of cells, intensity ++++)

Ki67 Labeling Index : Rt. breast: Ki-67 LI is about 20% in hotspot areas

HER2 Immunostaining : Rt. breast: Negative (score 0).

COMMENT:

* According to ASCO/CAP 2016 guidelines and the 2015 update, it is recommended to report hormone receptor status as either positive or negative with nuclear staining of >1% of tumor cells considered positive. Low ER positivity is defined as positive reaction in less than 1% of Ki67 staining is only indicated when ER is positive and HER2 is negative (Hammond et al. 2011; Slamon et al. 2013).

* Sensitivity for hormone receptor detection, on formalin-fixed paraffin-embedded specimens, ranges from 92-100%. As regards adjuvant tamoxifen therapy, ER has a positive predictive value of 55% and a negative predictive value of 47% (Petrovic et al. 1995).

* According to the joint ASCO/CAP 2016 guideline recommendations for HER2 testing and reporting, the status of HER2 should not be determined in all patients with invasive breast cancer. HER2 is reported to be positive (score 3+) when circumferential membrane staining is complete, intense is seen in > 10% of tumor cells. This staining should be readily appreciated, with a low-power objective and without concern for homogeneity and contiguous invasive cell population. The test is reported to be equivocal (score 2+) when a weak to moderate staining membrane staining observed in > 10% of tumor cells. Some breast carcinomas showing moderate to intense but incompletely circumferential membrane staining and others showing circumferential intense membrane IHC staining with <10% of tumor cells. Such cases can be either weak or strong (score 2+). The test is reported to be negative (score 1+) when incomplete membrane staining is faint, poorly perceptible, or even absent in tumor cells. The test is reported to be negative (score 0) when no staining is observed or membrane staining that is incomplete and faint, poorly perceptible and is seen in less than or equal 10% of tumor cells. When the result is equivocal, reflex testing (IHC on same specimen or ISH) on a new specimen is mandatory. The sector of HER2 negative cases that encompass cases with 1+ and cases of score 2+ membrane staining to be negative by ISH are suggested to be labeled as low HER2 expression as they may not be suitable for specific types of targeted therapy designed for these particular patients.

* The current consensus that breast carcinoma is categorized according to hormone status, HER2 overexpression and Ki67 labeling index into the following groups:

1. Luminal A: Positive ER and positive PR > 10% with negative HER2 and low Ki67 labeling index (< 14%).
2. Luminal B (HER -ve): Positive ER with negative HER2 and high Ki67 labeling index (> 14%) or Positive ER with negative HER2 and negative or low PR with any Ki67 labeling index.
3. Luminal B (HER2 +ve): Positive ER and positive HER2.
4. Non-luminal (HER2 enriched): Negative ER and PR with positive HER2.
5. Non-luminal (Triple negative TNF): Negative for all ER, PR and HER2. Subclassification of TNF cases into basal-like and non-basal-like and EGFR has no clinical significance to date (Goldhirsch, 2013).

Dr. Habiba Elfendy

Authentication Date: 24-02-2024



Patient's Name: Amal Fikri Mahmoud Moussa
Date: 2026-06-21
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH CONTRAST

TECHNIQUE: Multi slice axial CT images of the chest, abdomen and pelvis were obtained followed by multiplanar reconstruction of the source images. I.V contrast administration.

FINDINGS:

CECT CHEST:

- Bilateral MRM:
- No cavitory lesions or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

CECT ABDOMEN AND PELVIS:

- The liver is average in size showing homogenous CT density with no enhancing focal lesions.
- No biliary radicles dilatation.
- The gall bladder shows average wall thickness. Its cavity shows no dense stones.
- Normal CT appearance of the spleen, pancreas and both kidneys.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- No abdominal or pelvic pathologically enlarged lymph nodes.
- No free collection.

THE SCANNED SKELETON:

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

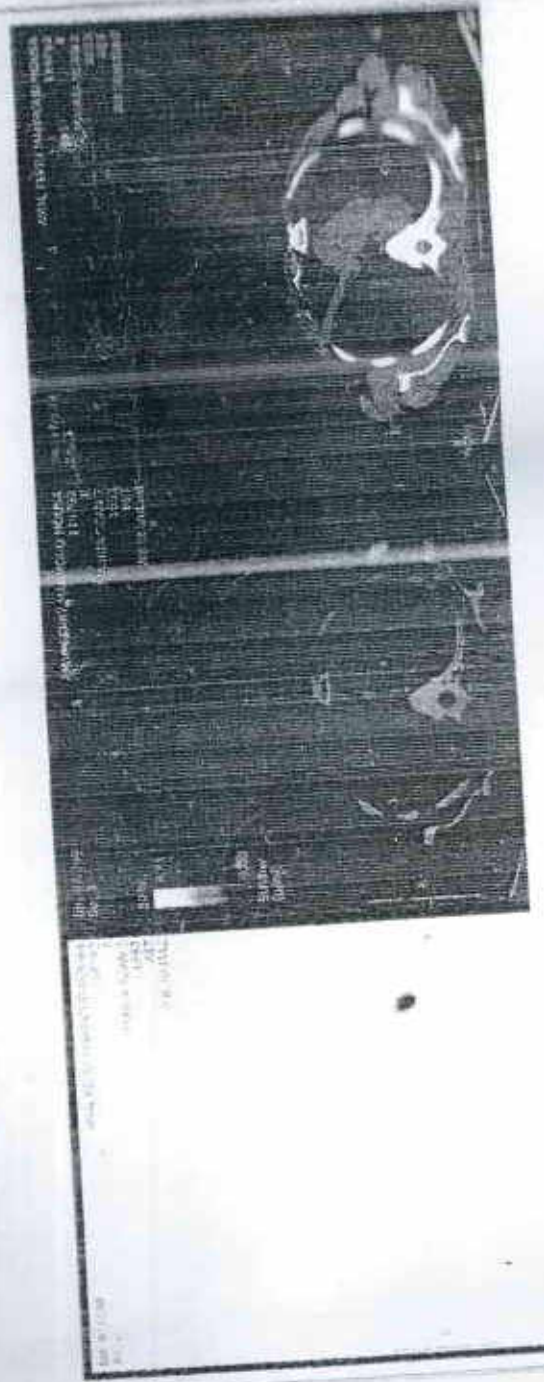
OPINION:

- Bilateral MRM.
- No hepatic lesions.
- No pulmonary nodules or masses.

Dr.Heba El Balsky.MD
Lecturer and Consultant of Diagnostic and Interventional Radiology,
NELI-Menoufia University

- Low grade metabolically active small right breast LIQ nodule measuring about 0.7 cm in max axial dimension with SUVmax 1.7

- Low grade metabolically active right axillary lymph nodes. The largest measuring about 1 cm in max axial dimension with SUVmax 1.4
- No metabolically active sizable pulmonary nodules, masses, infiltrations or bronchiectatic changes.
- No pleural or pericardial sac collection.
- No metabolically active grossly enlarged hilar or mediastinal lymph nodes.
- Normal tracer distribution at the myocardium



HEAD AND NECK

- No hypermetabolic lesions detected.
- No metabolically active grossly enlarged cervical lymph nodes.
- Patent nasopharyngeal and oro-pharyngeal air column.
- Normal appearance of the parotid and submandibular salivary glands detected on non contrast CT basis.
- No hypermetabolic gross brain lesions detected.
- Bilateral thyroid lobes partially calcified nodules with no pathological tracer uptake



CHEST

- Metabolically active left breast LIQ mass lesion measuring about 4 x 2.4 cm in max axial dimension with SUVmax 8.5
- Metabolically active left axillary and pectoral lymph nodes. The largest measuring about 2.9 cm in max axial dimension with SUVmax 8.

MUSCULO-SKELETAL

- *No hypermetabolic lesions detected.*

OPINION

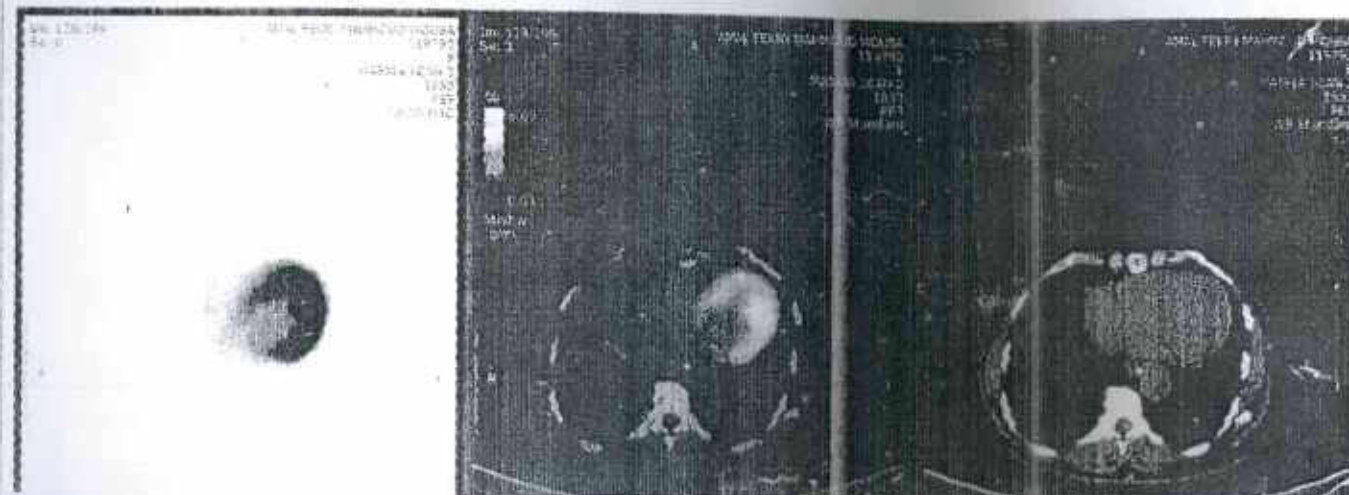
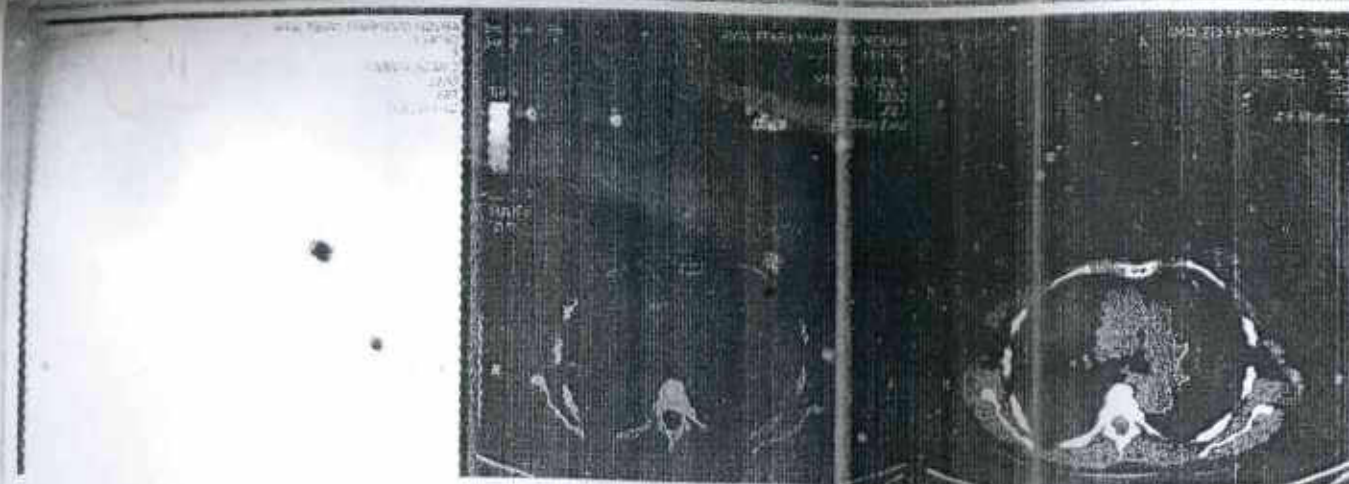
- *Metabolically active neoplastic left breast mass lesion with metabolically active left axillary and pectoral lymph nodes deposits for correlation with tumor markers and histopathology & immunophenotyping*
- *Low grade metabolically active small right breast nodule with low grade metabolically active right axillary lymph nodes, likely early / low grade neoplastic nodule with metastatic nodal deposits for correlation with histopathology & immunophenotyping*
- *No gross hypermetabolic lesions detected at the rest of the scanned body.*

Best Regards,
Prof. Dr. Maha Abd Elkareem
Professor of Nuclear Medicine
Kasr Al Ainy, Cairo University

For Further medical information or if you have any questions please do not hesitate to contact me

Dr. Ahmed Waheed
01005055422

Amal Fekry Mahmoud Mousa



ABDOMEN AND PELVIS:

- *No hypermetabolic lesions detected.*
- *No hypermetabolic hepatic focal lesions detected.*
- *No hypermetabolic pancreatic, splenic, suprarenal or renal focal lesions detected.*
- *No metabolically active grossly enlarged abdominal or pelvic lymph nodes.*
- *Aorta and IVC were unremarkable on non contrast CT basis.*
- *No ascites or pelvic collection.*

Amal Fekry Mahmoud Mousa

Pat.Name: امال فكري محمود

Pat.ID: 447349

Date: 23/09/2024

Pat. Age 65 Year(s)

Ref.Prof.Doc: Em Em

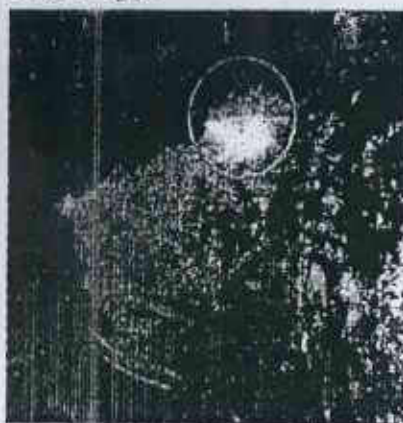
Thanks for reference

High Field Open (1.5T) Dynamic MR-Mammography of Both Breasts**Technique :**

- Axial T2
- Axial STIR
- Sagittal postcontrast 3D TFE of the right breast
- Sagittal postcontrast 3D TFE of the left breast
- Dynamic multiphase post-contrast study done in 8 minutes with MIP reconstruction.

Clinical background:**Pathologically proven carcinoma ,on neoadjuvant chemotherapy.****Compared to the last study dated 7-9-2024,the current study revealed:****Findings:****Right breast:**

- Extreme fibroglandular tissue
- Mild background parenchymal enhancement.
- Right breast few punctuate foci of parenchymal enhancement, the kinetic data obtained from them indicates a benign pattern, continuous steady rise not followed by wash out (type 1 curves).
- No areas of pathological enhancement along the mammographically detected right LOQ and retroareolar calcifications.
- No architecture distortion.
- No breast solid or cystic lesions.
- Normal skin thickness and contour of the right breast.
- There is multiple likely non-specific axillary lymph nodes.



Pat.Name: امال فكري محمود

Pat.ID: 447349

Date: 23/09/2024

Pat. Age 65 Year(s)

Ref.Prof.Doc: Em Em

Left breast:

- Extreme fibroglandular tissue
- Mild background parenchymal enhancement
- Left UIQ irregular shaped spiculated outline solid mass lesion is seen at 10-11 O'clock ,about 4.9 cm from the nipple and 1.6 cm from the chest wall.
 - It is seen measuring 37x12x24 mm along its maximum AP,IS and CC dimensions respectively.
 - It is seen reaching the subdermal region with fixation to the overlying skin.
 - Marked surrounding architectural distortion is noted.
 - It elicits low signal in T2WIs and STIR with heterogeneous enhancement in the post contrast series, the kinetic data obtained from the lesion indicates a malignant pattern ,rapid rise followed by washout ,type 3 curves
 - Restricted diffusion is noted within.
- Thickening of the skin of the left UIQ.
- Multiple pathologically enlarged level I and II axillary LNs are noted ,reaching 17x9 mm in diameter with restricted diffusion within and pathological enhancement in the post contrast series.

OPINION :

Mild bilateral mammary fibroadenosis .

Right breast BIRADS 2.

Left UIQ malignant looking solid mass lesion with skin fixation and axillary pathological lymphadenopathies ,categorized as BIRADS 6, based on the kinetic and morphological data.

N.B: Cardiomegaly is noted with ectatic ascending aorta ,for echocardiographic correlation.

Dr.S.Ali MD



Patient Name:	Amal Fekery Mahmoud	Patient Gender:	Female
Patient Code:	Q49790	DATE:	22-Jun-26 2:54:38 PM

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

Clinical history

- Female with history of bilateral breast cancer, for assessment.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post IV injection of 740 MBq of the tracer.

Findings

- No abnormal focal tracer uptake is seen that would count for metastatic osseous deposits.
- Heterogeneous tracer uptake is noted at the dorsal spine, likely degenerative changes, for clinical correlation.
- Tracer hold up in right kidney, for sonographic correlation.

Opinion

- No convincing scintigraphic evidence of metastatic osseous deposits.

Dr. Aafaa M. Solaiman

Prof. Ahmed Kandeel, MD

PCL XL Error
Subsystem:
Error:
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273



٢٠٢٩/٠٣ ٢٥٩.٢٠٩.٠١.٠٣٤٧

بدون عمل

أنثى مسنونة أنثى



البطاقة سارية حتى ٢٠٢٨/٠٣/٢٢



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inputReaderError
ReadImage
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09/16/2026

MDT FORM

Name:	اسم المريضة	MRN:	10885
Age:	68 Y	ID:	25902060100347
Menopausal Sta:	post menopausal	Phone: #	01126291984
NRS SCORE:		Marital Status:	
BMT:		Family History:	
Gender:		Reason Of MDT:	
CO Morbidity:		new case ,, on adj ET	
Surgical History:*	Bil MRM 11/2024	ECOG Performance Status	0 1 2 3 4

Clinical Data:

BX: Mixed IDC, ILC G2, Same behaviour bilateral

ER: +ve 100% **PR:** 60% **HER2:** 0 **KI67:** 30%

History:

Bilateral Breast cancer (Synchronous) Rt cT1N1/ Lt cT3N3/Luminal B/
Received NACT ACx4 , TC x4 then Underwent Bil MRM--> PORT bil ,,
Now on adjuvant hormonal tt (completed 2y of Femara)
Wishes to continue at our Institute

Sonomammography:

Biomarker/Molecular

Mets Workup:

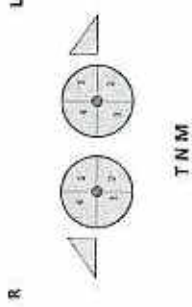
CT CAP in 6/2026 : free

Recommendations:

To continue adj Femara

MDT Outcome & Plan

ADJ FEMARA



Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
	د. محمد نوردي		

[illegible]

E-Mourad E. S.

اسماء بنت عبد مناف

[illegible]

محول من ا د

53624500640 رقم الترخيص

HER2 and HER2 using

sitive (100%) of ex

tive 160% of cell

precursor: Ki-67 L1 in

L. brevis: Nequa

2015 update: 1) a 'return to work' is considered positive. Low E and HER2 is negative. (Hamm)

of 15.5% and a negative pre-

HEB is reported to be co-

on The test is reported to 105

membranes (Hilgendorf and

When the machine is on, when the spinning is done, the machine will stop.

Minor cells. When the cell is negative, cells that are

How HEED affects you this

anyone who has ever been in a car accident knows that the insurance company will try to get you to sign a release that says you are not injured and that you will not sue them. This is a very common tactic used by insurance companies to avoid paying for the damages caused by their negligence. If you are involved in a car accident, it is important to consult with a lawyer before signing any release or accepting any settlement offer from the insurance company. A lawyer can help you understand your rights and ensure that you are not being taken advantage of by the insurance company.

MEET and sign up at www.rockon.com

DR with positive $\text{H}^1(\mathbb{R}^n, \mathbb{R})$

ALLER, C. A. AND HERR, S. J.
1993. 2013.

Figure 1. Schematic representation of the experimental design. The subjects were divided into two groups: the control group (CG) and the experimental group (EG). The CG was divided into two subgroups: the control group (CG) and the control group (CG). The EG was divided into two subgroups: the experimental group (EG) and the experimental group (EG). The CG was divided into two subgroups: the control group (CG) and the control group (CG). The EG was divided into two subgroups: the experimental group (EG) and the experimental group (EG).

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أ.د. هادي السماعيل - أ.د. بركات تاجور
د. غيبة الكندي - د. حي جاد - د. ناسي
د. مصطفى أحمد - د. محمد يسري - د. مكي
د. محمد المشاوي - د. أحمد علي شماتة

رئيسة الوحدة
أ.د. محمد أكرم نوح

بالتأكيد..

معامل
البرج

Pathology Unit

5287 65 :السن

فرع : B-Mourad

تاريخ الحضور : 10-02-2024

إسم المريض : أمال فكري سعيد RT.B

تاريخ الاستلام : 12-02-2024

محول من أ د : محمد لطيف

رقم الزيارة : 53624500510

NATURE OF SPECIMEN:

Bilateral breast U/S guided core biopsies, for highly suspicious breast solid lesions, BIRDS 5.

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Two biopsies were received:

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DIAGNOSIS:

(1) and (2) Bilateral breast masses, US-guided core biopsies: Mixed invasive carcinoma of no special type, NST (= Invasive duct carcinoma) and lobular carcinoma, grade 2.

Neveen Tahoun

Cherif

Prof. Dr. Neveen S. Tahoun

Dr. Ahmed Ali Shehata

Authentication Date: 12-02-2024



המחלקה
המרכזית

R-Mourad E.

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53624520673 8.11.19

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Dr. Mahiba Elfendy

C199T

Patient's Name: Amal Fikri Mahmoud Moussa
Date: 2026-06-21
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH CONTRAST

TECHNIQUE: Multi slice axial CT images of the chest, abdomen and pelvis were obtained followed by multiplanar reconstruction of the source images. I.V contrast administration.

FINDINGS:

CECT CHEST:

- Bilateral MRM.
- No cavitary lesions or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

CECT ABDOMEN AND PELVIS:

- The liver is average in size showing homogenous CT density with no enhancing focal lesions.
- No biliary radicles dilatation.
- The gall bladder shows average wall thickness. Its cavity shows no dense stones.
- Normal CT appearance of the spleen, pancreas and both kidneys.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- No abdominal or pelvic pathologically enlarged lymph nodes.
- No free collection.

THE SCANNED SKELETON:

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-ossseous soft tissue component.

OPINION:

- Bilateral MRM.
- No hepatic lesions.
- No pulmonary nodules or masses.

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